

Australian MMI Question Bank — FREE SAMPLE

Original practice material for Australian medical and dental school MMIs. Not affiliated with any university, UCAT ANZ, GAMSAT, or admissions body. For general practice only.

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Ethics & Professionalism

You are a student on clinical placement. You overhear a nurse criticising a patient loudly in the corridor, using unprofessional language about the patient's weight.

Task: You are about to enter the lift with this nurse. What do you do, and what principles guide your response?

✓ THINKING FRAMEWORK

1. Assess immediate risk — is the patient harmed or distressed right now? 2. Address the nurse respectfully and privately, not in public confrontation. 3. Distinguish between immediate duty (patient dignity) and reporting duty (professional standards). 4. Use the principles: respect for dignity, non-maleficence, professional accountability. 5. Escalate through appropriate channels if the behaviour continues (clinical educator, supervisor).

△ COMMON PITFALLS

- Confronting the nurse loudly in front of others (escalates conflict, embarrasses both)
- Doing nothing ('it's not my place') — passive complicity
- Jumping straight to formal reporting without addressing it once
- Focusing only on the nurse and forgetting the patient's perspective

→ FOLLOW-UP QUESTIONS

- The nurse defends their behaviour, saying the patient 'doesn't care'. How do you respond?
- What if the nurse is your direct supervisor?
- When is it appropriate to formally report vs. informally address?

Communication & Role-Play

ROLE-PLAY: You are a dental student and a patient (played by the examiner) has just been told they need a tooth extraction. The patient is anxious and says: 'I really don't want this. Isn't there another way? I'm scared of the pain.'

Task: Respond to the patient in the role-play. Address their concern and build trust.

✓ THINKING FRAMEWORK

1. Acknowledge the emotion first ('I can see you're really anxious about this — that's completely understandable'). 2. Explore the source of fear (pain, needles, loss of tooth, past bad experience). 3. Explain options honestly (root canal, alternatives, why extraction may be indicated). 4. Explain pain management (local anaesthetic, gentle technique, post-op care). 5. Involve them in the decision — it's their mouth.

△ COMMON PITFALLS

- Jumping straight to clinical justification without acknowledging fear
- Making promises you can't keep ('it won't hurt at all')
- Minimising the patient's feelings ('it's not that bad')
- Using jargon the patient doesn't understand

→ FOLLOW-UP QUESTIONS

- The patient says a relative had a horrible extraction. How does this change your approach?
- The patient agrees but asks what happens 'if it goes wrong'. Respond.
- How do you communicate risk honestly without frightening the patient further?

Motivation & Personal Insight

Why do you want to study [dentistry/medicine]? What is your genuine motivation?

Task: Answer honestly, with a story or example.

✓ THINKING FRAMEWORK

1. Be specific to YOU — a story, experience, or person that shaped you. 2. Connect it to what the profession actually involves (not just 'helping people' generically). 3. Show you've considered the reality: the study load, the patient relationships, the daily realities. 4. Acknowledge a challenge you've seen or anticipate. 5. End with what you'll contribute.

△ COMMON PITFALLS

- Generic answers ('I love helping people' / 'job security')
- Sounding rehearsed or reciting a script
- Not mentioning any understanding of the profession's realities
- Avoiding any self-reflection

→ FOLLOW-UP QUESTIONS

- What do you think is the hardest part of being a dentist/doctor?
- What will you do if you don't get in this year?
- Why this school/university specifically?

Dental-Specific Scenario

A patient with significant dental anxiety and a strong gag reflex needs a filling. They've postponed treatment for years and are only here now because of pain.

Task: How do you manage this patient?

✓ THINKING FRAMEWORK

1. Acknowledge and validate the anxiety — this took courage. 2. Build trust slowly; don't rush to the chair. 3. Discuss options: local anaesthetic techniques, desensitisation, sedation options (nitrous oxide/IV sedation where indicated), breaks during treatment. 4. Agree a 'stop signal' — give the patient control. 5. Consider a staged approach (build up over visits).

△ COMMON PITFALLS

- Dismissing the gag reflex as 'manageable' without a plan
- Scheduling a full filling without preparing the patient
- Ignoring the years of avoidance and the fear behind it
- Not offering control mechanisms (stop signal, breaks)

→ FOLLOW-UP QUESTIONS

- The patient is fine at the first visit but anxious at the next. How do you maintain momentum?
- How do you balance patient comfort with treatment efficiency?
- What does a 'patient-centred' approach to anxiety look like?

Your supervisor gives you critical feedback on your clinical performance that you feel is unfair.

Task: How do you respond and what do you do?

✓ **THINKING FRAMEWORK**

1. Listen fully first — don't defend in the moment. 2. Ask clarifying questions to understand the specific concern. 3. Separate the emotion (feeling unfair) from the content (is there a valid point?). 4. Reflect before responding if needed ('let me think about that and come back to you'). 5. Act on what's valid, and if truly incorrect, raise it professionally later.

△ **COMMON PITFALLS**

- Getting defensive immediately
- Accepting everything without reflection (no self-advocacy)
- Dwelling on the 'unfairness' and ignoring the learning
- Failing to follow up or demonstrate improvement

→ **FOLLOW-UP QUESTIONS**

- What's the difference between defensiveness and self-advocacy?
- How do you ask for feedback proactively?
- Describe a time feedback changed your practice.